

BETH ISRAEL GENERAL HOSPITAL
Department of Internal Medicine · 330 Brookline Ave, Boston, MA 02215

PATIENT	Margaret Sullivan	MRN	MRN-0094821
DOB	September 12, 1958 (Age 67)	SEX	Female
ADMIT DATE	August 5, 2026	DISCHARGE DATE	August 9, 2026
ATTENDING	Dr. Priya Nair, MD	INSURANCE	Medicare Part A/B

DISCHARGE SUMMARY

CHIEF COMPLAINT

Acute onset chest pain radiating to left arm, diaphoresis, and shortness of breath for approximately 3 hours prior to presentation.

HISTORY OF PRESENT ILLNESS

Margaret Sullivan is a 67-year-old female with a past medical history significant for type 2 diabetes mellitus (A1C 9.1%), hypertension, hyperlipidemia, and a 30-pack-year smoking history (quit 2018) who presented to the emergency department via EMS with acute onset substernal chest pain 8/10 in severity, radiating to the left arm and jaw, associated with diaphoresis and dyspnea. Initial 12-lead ECG demonstrated ST-elevation in leads II, III, and aVF, consistent with inferior STEMI. Patient was taken emergently to the cardiac catheterization laboratory.

VITAL SIGNS ON ADMISSION

BP	162/98 mmHg	HR	104 bpm	RR	22/min
SpO2	91% (room air)	Temp	98.9°F	Weight	84 kg

DIAGNOSTIC RESULTS

ECG: ST-elevation leads II, III, aVF — inferior STEMI confirmed.
Troponin I (peak): 18.4 ng/mL (HIGH — normal <0.04)
BNP: 620 pg/mL (HIGH)
HbA1c: 9.1% (HIGH — uncontrolled diabetes)
LDL: 148 mg/dL (HIGH)
Creatinine: 1.2 mg/dL (within normal limits)
Echocardiogram: EF 40%, inferior wall hypokinesis, mild mitral regurgitation.

DIAGNOSES

- ST-Elevation Myocardial Infarction (STEMI) — inferior wall
- Coronary artery disease — right coronary artery 95% occlusion
- Acute systolic heart failure (EF 40%)
- Type 2 diabetes mellitus, poorly controlled (HbA1c 9.1%)
- Hypertension
- Hyperlipidemia
- Mild mitral regurgitation

HOSPITAL COURSE

Patient underwent emergent percutaneous coronary intervention (PCI) with placement of a drug-eluting stent to the right coronary artery with TIMI 3 flow restored. Post-procedure, patient was monitored in the cardiac ICU for 48 hours. Aspirin 325 mg and ticagrelor 90 mg BID initiated for dual antiplatelet therapy. IV metoprolol transitioned to oral. Lisinopril initiated for ACE inhibition post-MI. Atorvastatin increased to 80 mg. Endocrinology consulted — insulin glargine 12 units at bedtime initiated for glycemic control. Patient remained hemodynamically stable and was transferred to the step-down unit on day 3.

DISCHARGE MEDICATIONS

Medication	Dose	Route	Frequency
Aspirin	81 mg	oral	daily
Ticagrelor (Brilinta)	90 mg	oral	twice daily
Metoprolol succinate	50 mg	oral	daily
Lisinopril	5 mg	oral	daily
Atorvastatin	80 mg	oral	at bedtime
Insulin glargine	12 units	subcutaneous	at bedtime
Nitroglycerin SL	0.4 mg	sublingual	PRN chest pain

FOLLOW-UP INSTRUCTIONS

- Cardiology clinic in 5 days — repeat echocardiogram, titrate lisinopril
- Primary care in 2 weeks — recheck BMP, lipid panel, HbA1c
- Cardiac rehabilitation program — referral placed
- DO NOT stop ticagrelor or aspirin without cardiology approval (stent thrombosis risk)
- Return to ED immediately for: chest pain, shortness of breath, palpitations, syncope

Electronically signed by: **Dr. Priya Nair, MD** — Board Certified Internal Medicine & Cardiology
Date: August 9, 2026 · Beth Israel General Hospital